

EXERCISES AND EXACT SOLUTIONS

Referrals That Do Not Lose the Patient

Independent practice, facilitated discussion, and assessment reference

Production Unit	PU-B07-C30
Notebook	V7_B_N10
Version	0.1
Assessment boundary	Learning evidence—not operational authorization

Instructions

Complete Part A before consulting the solutions. State assumptions explicitly. Where evidence is insufficient, write an abstention or escalation response rather than inventing precision.

Exercise 1

Add network edges.

- Required output: a concise decision-ready response with owner, evidence, uncertainty, and boundary where applicable.

Exercise 2

Separate urgent cases.

- Required output: a concise decision-ready response with owner, evidence, uncertainty, and boundary where applicable.

Exercise 3

Protect privacy.

- Required output: a concise decision-ready response with owner, evidence, uncertainty, and boundary where applicable.

Exact solutions and scoring guide

Solution 1

Build directed sending-receiving flows with counts, delays, capacity, and completion.

Solution 2

Stratify by authorized urgency definition and case mix; never infer urgency from delay alone.

Solution 3

Aggregate routes, minimize identifiers, control access, log use, and suppress small flows.

Scoring rubric

Criterion	Strong evidence	Revision required
Decision framing	Owner, action, horizon, authority named	Model output substituted for decision
Evidence	Concepts, grain, provenance, limits explicit	Data treated as self-explanatory
Uncertainty	Abstention/escalation used appropriately	False precision or hidden limitations
Governance	Human authority and safeguards retained	Authority silently delegated